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IRBESARTAN MY ALL STRENGTHS FCT PIL



GERNERAL INFORMATION

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REGULATORY APPROVAL

BESARTIN Film Coated Tablets 150mg

BESARTIN Film Coated Tablets 300mg

IRBESARTAN

lotus
PHARMACEUTICAL

pharmacode

Product Name

Besartin Film Coated Tablets 150 mg
Besartin Film Coated Tablets 300 mg

Name and Strength of Active Substance(s)

Each Besartin film coated tablet 150 mg contains 150 mg of irbesartan.
Each Besartin film coated tablet 300 mg contains 300 mg of irbesartan.

Product Description

Besartin film coated tablet 150 mg: White to off-white, oblong, biconvex film-coated tablets with score line and 'G' embossed on one side and plain on the other.
Besartin film coated tablet 300 mg: White to off-white, oblong, biconvex film-coated tablets with score line.

Pharmacodynamics

Irbesartan is a potent, orally active, selective angiotensin-II receptor (type AT1) antagonist. It is expected to block all actions of angiotensin-II mediated by the AT1 receptor, regardless of the source or route of synthesis of angiotensin II. The selective antagonism of the angiotensin-II (AT1) receptors results in increases in plasma-renin levels and angiotensin II levels, and a decrease in plasma aldosterone concentration. Serum potassium levels are not significantly affected by irbesartan alone at the recommended doses.

Clinical Efficacy: Hypertension: Irbesartan lowers blood pressure with minimal change in heart rate. The decrease in blood pressure is dose-related for once-daily doses with a tendency towards plateau at doses > 300 mg. Peak reduction of blood pressure is achieved within 3-6 hours after administration and the blood pressure-lowering effect is maintained for at least 24 hours. At 24 hours, the reduction of blood pressure was 60-70% of the corresponding peak diastolic and systolic responses at the recommended doses.

Once-daily dosing with 150 mg produced trough and mean 24-hour responses similar to twice-daily dosing on the same total dose. The blood pressure-lowering effect of irbesartan is evident within 1-2 weeks, with the maximal effect occurring by 4-6 weeks after start of therapy. The antihypertensive effects are maintained during long-term therapy. After withdrawal of therapy, blood pressure gradually returns toward baseline. Rebound hypertension has not been observed.

Hypertension and Type 2 Diabetes with Renal Disease: Irbesartan decreases the progression of renal disease in patients with chronic renal insufficiency and overt proteinuria. The effects of irbesartan on microalbuminuria in hypertensive patients with type 2 diabetes mellitus shows that irbesartan 300 mg delays progression to overt proteinuria in patients with microalbuminuria.

Pharmacokinetics

Absorption

After oral administration, irbesartan is well absorbed; absolute bioavailability approximately 60-80%. Concomitant food intake does not significantly influence the bioavailability of irbesartan.

Distribution

Plasma protein-binding is approximately 96%, with negligible binding to cellular blood components. The volume of distribution is 53-93 L. Peak-plasma concentrations are attained at 1.5-2 hours after oral administration.

Metabolism

Irbesartan is metabolized by the liver via glucuronide conjugation and oxidation. The major circulating metabolite is irbesartan glucuronide (approximately 6%). Irbesartan is primarily oxidized by the cytochrome P-450 enzyme CYP2C9; isoenzyme CYP3A4 has negligible effect.

Excretion

The total body and renal clearances are 157-176 and 3-3.5mL/min, respectively. The terminal elimination half-life of irbesartan is 11-15 hours. Steady-state plasma concentrations are attained within 3 days after initiation of a once-daily dosing regimen.

Special Population:

Elderly: No dosage adjustment is necessary in elderly patients.

Renal Impairment: In patients with renal impairment or those undergoing haemodialysis, the pharmacokinetic parameters of irbesartan are not significantly altered. Irbesartan is not removed by haemodialysis.

Hepatic Impairment: In patients with mild to moderate cirrhosis, the pharmacokinetic parameters of irbesartan are not significantly altered.

Indications

Treatment of essential hypertension.

Treatment of diabetic nephropathy with an elevated serum creatinine and proteinuria (>300 mg/day) in patients with type 2 diabetes and hypertension.

In this population, irbesartan reduces the rate of progression of nephropathy as measured by the occurrence of doubling of serum creatinine or end-stage renal disease (need for dialysis or renal transplantation).

Recommended Dosage

The usual recommended initial and maintenance dose is 150 mg once daily. May be taken with or without food. Initiation of therapy with 75 mg could be considered, particularly in haemodialysed patients and in the elderly > 75 years. In patients insufficiently controlled with 150 mg once daily, the dose of irbesartan can be increased to 300 mg or other antihypertensive agents can be added.

Hypertensive Type 2 Diabetic Patients: Starting dose should be 150 mg titrated up to 300 mg once daily as preferred maintenance dose for treatment of renal disease. Renal Impairment: No dosage adjustment is necessary in patients with impaired renal function. A lower starting dose (75 mg) should be considered for patients undergoing haemodialysis.

Hepatic Impairment: No dosage adjustment is necessary in patients with mild to moderate hepatic impairment.

Children: Irbesartan is not recommended for use in children and adolescents.

Mode of Administration

Oral

Contraindications

Hypersensitivity to the active substance, or to any of the excipients.

Second and third trimesters of pregnancy.

Warnings and Precautions:

Intravascular volume depletion: Symptomatic hypotension, especially after the first dose, may occur in patients who are volume and/or sodium depleted by vigorous diuretic therapy, dietary salt restriction, diarrhoea or vomiting. Such conditions should be corrected before the administration of Irbesartan.

Renovascular hypertension: There is an increased risk of severe hypotension and renal insufficiency when patients with bilateral renal artery stenosis or stenosis of the artery to a single functioning kidney are treated with medicinal products that affect the renin-angiotensin-aldosterone system. While this is not documented with Irbesartan, a similar effect should be anticipated with angiotensin-II receptor antagonists.

Renal impairment and kidney transplantation: When Irbesartan is used in patients with impaired renal function, a periodic monitoring of potassium and creatinine serum levels is recommended.

Hyperkalaemia: As with other medicinal products that affect the renin-angiotensin-aldosterone system, hyperkalaemia may occur during the treatment with Irbesartan, especially in the presence of renal impairment, overt proteinuria due to diabetic renal disease, and/or heart failure. Close monitoring of serum potassium in patients at risk is recommended.

Lithium: The combination of lithium and Irbesartan is not recommended.

Aortic and mitral valve stenosis, obstructive hypertrophic cardiomyopathy: as with other vasodilators, special caution is indicated in patients suffering from aortic or mitral stenosis, or obstructive hypertrophic cardiomyopathy.

Primary aldosteronism: Patients with primary aldosteronism generally will not respond to antihypertensive medicinal products acting through inhibition of the renin-angiotensin system. Therefore, the use of Irbesartan is not recommended.

Dual blockade of the renin-angiotensin-aldosterone system

(RAAS): Concomitant use of ACE-inhibitors, angiotensin II receptor blockers or aliskiren increases the risk of hypotension, hyperkalaemia and decreased renal function (including acute renal failure). Dual blockade of RAAS through the combined use of ACE-inhibitors, angiotensin II receptor blockers or aliskiren is therefore not recommended. If dual blockade therapy is considered absolutely necessary, this should only occur under specialist supervision and subject to frequent close monitoring of renal function, electrolytes and blood pressure. ACE-inhibitors and angiotensin II receptor blockers should not be used concomitantly in patients with diabetic nephropathy. General: In patients whose vascular tone and renal function depend predominantly on the activity of the renin-angiotensin-aldosterone system (e.g. patients with severe congestive heart failure or underlying renal disease, including renal artery stenosis), treatment with angiotensin converting enzyme inhibitors or angiotensin-II receptor antagonists that affect this system has been associated with acute hypotension, azotaemia, oliguria, or rarely acute renal failure. As with any antihypertensive agent, excessive blood pressure decrease in patients with ischaemic cardiopathy or ischaemic cardiovascular disease could result in a myocardial infarction or stroke. As observed for angiotensin converting enzyme inhibitors, irbesartan and the other angiotensin antagonists are apparently less effective in lowering blood pressure in black people than in non-blacks, possibly because of higher prevalence of low-renin states in the black hypertensive population. Pregnancy: Angiotensin II Receptor Antagonists (AIIAs) should not be initiated during pregnancy. Unless continued AIIA therapy is considered essential, patients planning pregnancy should be changed to alternative antihypertensive treatments which have an established safety profile for use in pregnancy. When pregnancy is diagnosed, treatment with AIIAs should be stopped immediately, and, if appropriate, alternative therapy should be started. Lactose: This medicinal product contains lactose. Patients with rare hereditary problems of galactose intolerance, the Lapp lactase deficiency or glucose-galactose malabsorption should not take this medicinal product. Paediatric patients: Irbesartan has been studied in paediatric populations aged 6 to 16 years old but the current data are insufficient to support an extension of the use in children until further data become available.

Interactions

Diuretics and Other Antihypertensive Agents: Other antihypertensive agents may increase the hypotensive effects of irbesartan. However, irbesartan has been safely administered with other antihypertensive agents eg, beta-blockers, long-acting calcium channel blockers and thiazide diuretics. Prior treatment with high-dose diuretics may result in volume depletion and a risk of hypotension when initiating therapy with irbesartan (see Warning and Precautions). Potassium Supplements and Potassium-Sparing Diuretics: Concomitant use of potassium-sparing diuretics, potassium supplements, salt substitutes containing potassium or other drugs that may increase serum potassium levels (eg, heparin) may lead to increases in serum potassium and is therefore, not recommended (see Warning and Precautions). Nonsteroidal Anti-Inflammatory Drugs (NSAIDs): When angiotensin II antagonists are administered simultaneously with NSAIDs (ie, selective COX-2 inhibitors, acetylsalicylic acid (>3 g/day) and nonselective NSAIDs), attenuation of the antihypertensive effect may occur. Concomitant use of angiotensin II antagonists and NSAIDs may lead to an increased risk of worsening of renal function, including possible acute renal failure, and an increase in serum potassium, especially in patients with poor pre-existing renal function. The combination should be administered with caution, especially in the elderly. Patients should be adequately hydrated and consideration should be given to monitoring renal function after initiation of concomitant therapy and periodically thereafter. Lithium: Reversible increases in serum lithium concentrations and toxicity have been reported during concomitant administration of lithium with angiotensin converting enzyme inhibitors. Similar effects have been very rarely reported with irbesartan so far. Therefore, this combination is not recommended (see Special warnings and precautions for use). If the combination proves necessary, careful monitoring of serum lithium levels is recommended. Additional Information on Drug Interactions: Irbesartan is mainly metabolized by CYP2C9 and to a lesser extent by glucuronidation. No significant pharmacokinetic or pharmacodynamic interactions were observed when irbesartan was co-administered with warfarin, a drug metabolized by CYP2C9. The pharmacokinetics of digoxin

was not altered by co-administration of irbesartan.

Statement on Usage During Pregnancy and Lactation

Pregnancy
It is not recommended during the first trimester of pregnancy and is contraindicated during the second and third trimesters of pregnancy.
Lactation
Available evidence and/or expert consensus is inconclusive or is inadequate for determining infant risk when used during breastfeeding. Weigh the potential benefits of drug treatment against potential risks before prescribing this drug during breastfeeding.

Undesirable Effects

Investigations: hyperkalaemia, increases in plasma creatine kinase
Cardiac disorders: tachycardia
Vascular disorders: orthostatic hypotension, flushing
Nervous system disorders: headache, dizziness
Respiratory, thoracic and mediastinal disorders: cough
Gastrointestinal disorders: nausea, vomiting, diarrhoea, dyspepsia/heartburn, dysgeusia
General disorders and administration site conditions: fatigue
Renal and urinary disorders: impaired renal function including cases of renal failure in patients at risk
Skin and subcutaneous tissue disorders: urticaria, leukocytoclastic vasculitis
Musculoskeletal and connective tissue disorders: arthralgia, myalgia (in some cases associated with increased plasma creatine kinase levels), muscle cramps
Immune system disorders: hypersensitivity reactions such as angioedema, rash, urticaria
Hepato-biliary disorders: hepatitis, abnormal liver function, cholestasis, jaundice
Hematologic disorders: thrombocytopenia
Reproductive system and breast disorders: sexual dysfunction
Ear and labyrinth disorders: tinnitus

Overdose and Treatment

Limited information is available with regard to overdose in humans. The most prominent manifestations of irbesartan overdose were hypotension and tachycardia, bradycardia also occurred. If symptomatic hypotension should occur, supportive treatment should be instituted. Irbesartan is not removed by haemodialysis.

Storage Conditions

Store below 30°C

Dosage Forms and Packaging Available

Each PVC/PVDC/Aluminium blister contains 14 tablets
Pack size of 28 tablets per carton box (2 blisters x 14 tablets)

Manufacturer

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Product Registration Holder

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30/10/2025